SMART CARE 360

AI-guided Patient Intake

Explore how AI streamlines pre-visit **medical history** intake, enhancing provider productivity and patient outcomes

Transforms Patient Histories

Transforming patient intake with **AI-powered**, evidence-based solutions that streamline medical history capture and workflow efficiency for hospitals and clinics

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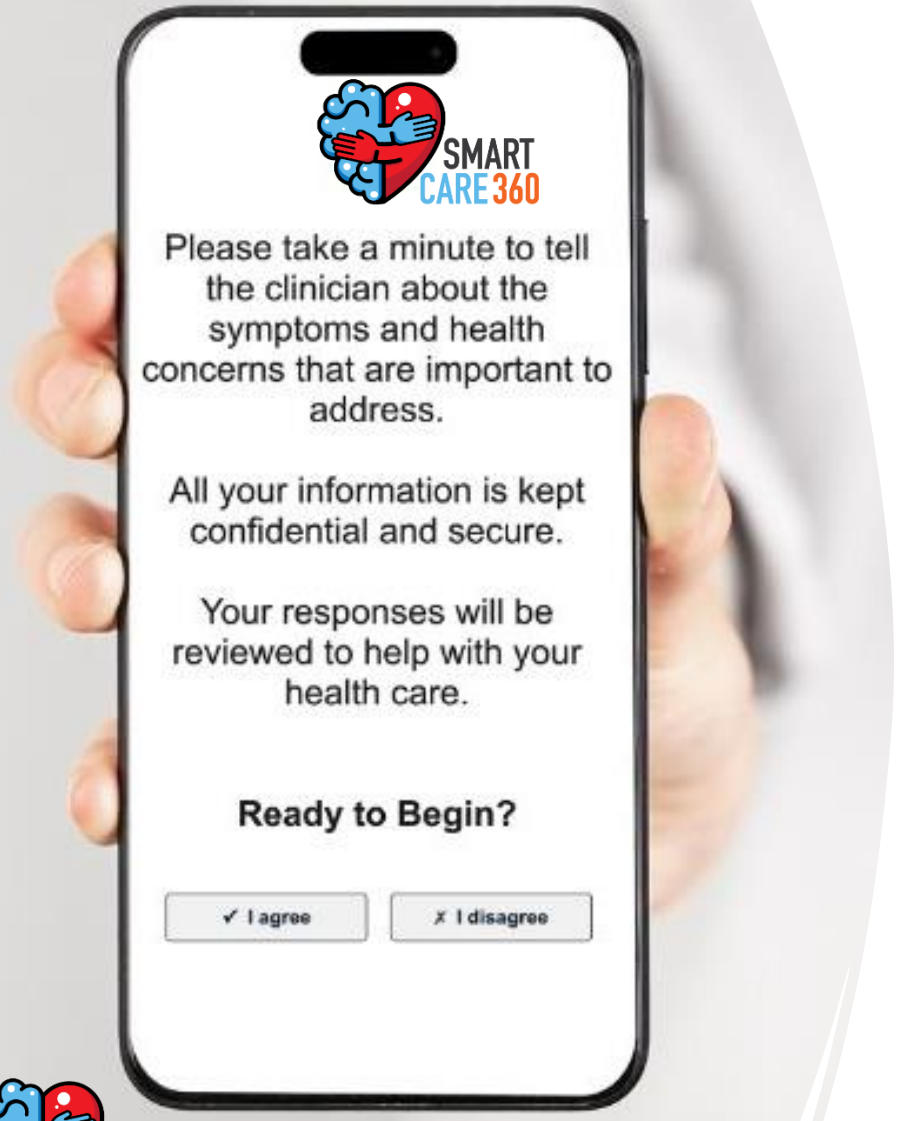

Precise Information, Fast

The intelligent platform delivers a faster, more **efficient workflow** for providers and a convenient patient experience with **shorter wait times**.

FACT: MDs using SmartCare360 save 25-50% of the time typically spent on documentation, enabling more focus on patient care and outcomes

Integrates Easily

Generates a **detailed and editable medical note.** Seamless integration with **ECW**, **Cerner**, **Epic**, and expanding to more EMRs everyday.

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Standardizes Data Capture

Effortlessly captures detailed information.

Boosts reimbursement and compliance

(eg. **Social Determinants of Health (SDOH)** and other **CMS codes.**

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SmartCare360 Addresses Real-World Issues

Increasing Patient Volumes: Healthcare providers are under pressure to see more patients due to growing demand, leading to longer hours and potential burnout.

Administrative Burden: Clinicians spend significant time on non-clinical tasks like documentation, coding, and billing, reducing time for patient care.

Fragmented Data: Patient information is often scattered across multiple systems, making it difficult to access and integrate data for informed decision-making.

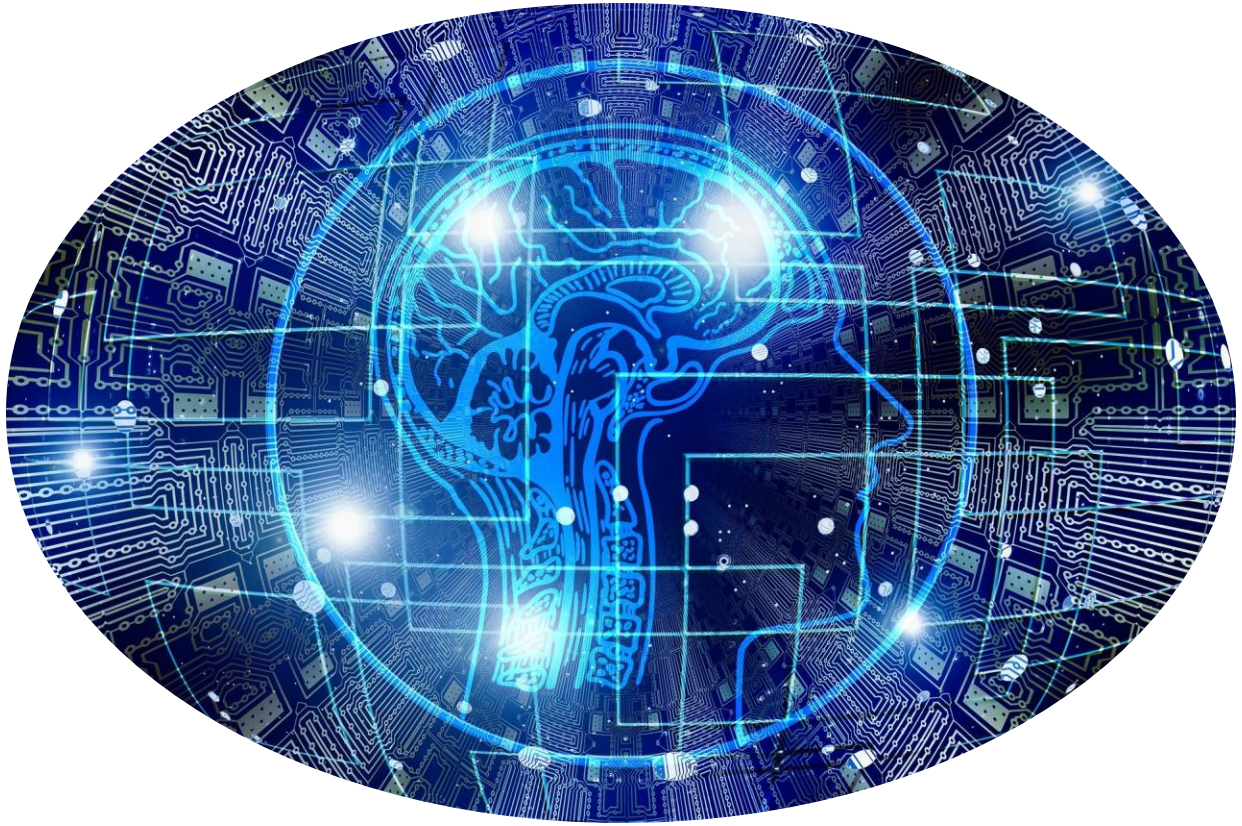
Improved Productivity Outcomes & Quality

Addressing issues like increasing patient volumes, administrative burn out and fragmented data is crucial to improving productivity, reducing clinician burnout, and delivering high-quality patient care.

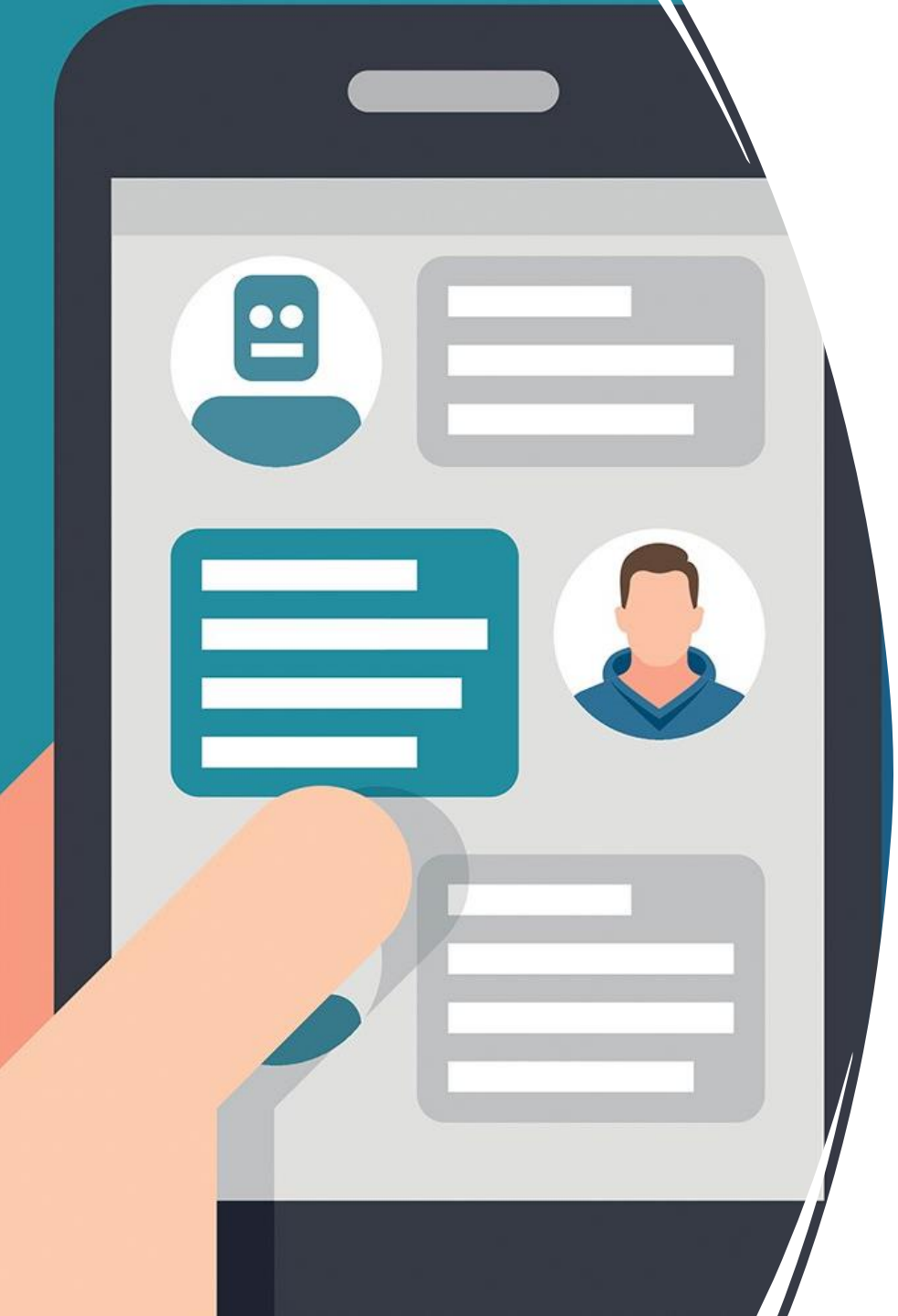
FACT: SmartCare360 Patients love us: + 90% patient satisfaction score

SmartCare360 Solution

Our adaptive, **AI-guided** pre-visit interview modules collect highly relevant information for clinicians, digitizing the process.



FACT: SmartCare360 can have your clinic up and running in less than 24 hours.



Not a form
Not a Chatbot

Not a form or chatbot; we've codified the physician's "brain"; by leveraging advanced computing.

Output informs triage, coding, and data-informed medical care.

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How Does it Work?

Clinic sends text message, patient answers adaptive questions, EMR receives synopsis. Synopsis is a detailed medical history; with scores, EB tools to support GDMT, triage.

Text message sent to patient with link to complete medical history

Patient completes web-based adaptive SmartCare360 intake

Clinician reviews structured SmartCare360 intake in their EMR

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Instant Value

Informs **care delivery** & **navigation**, enhances **workflows**, provides **evidence-based data sets**, and resource management.

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Growing Global Reach & Impact

Clients in **US**, **Canada**, and **Middle East**; growing **SME network**, **telehealth platforms**, **hospitals**, demonstrated value and efficiencies in **Virtual Care**

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SmartCare 360 Value

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Saves Clinicians Time

60-80% of a visit is entering patient data

MDs using SmartCare360 **save time (25-50%)**

5 minutes saved per patient (average)

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Make GDMT Easy

13% of HF patients get the medications they need for GDMT

SmartCare360 highlights if patients are on track

Identifies personalized GDMT optimization

Visual cues flag important gaps

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Reduce Costs & Boost Revenue

$125 Billion lost **every year** due to inaccurate billing & coding

Passive data capture

Improve **accuracy**, completeness

Demonstrate **compliance** with **CMS** standards

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Enhance Clinician Time

Frees MDs to focus on clinical.

Enhanced ability to evaluate, analyze, and plan patient diagnostics and treatment.

Reduced MD burnout.

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Structured Clinical History Note

Easy to read medical notes that convert the patient story into a concise, standard layout.

Generated pre-visit

Reduces errors in medication profiles

Interoperable with any system

Name: **Sara**
Sex: **Female**
Age: **25**
CC: **Headache(s) x 5 Days**

Time: **23:15pm, Jun. 4, 2023**
Duration: **4min 19sec**

Symptoms

- **Headache(s)**
  - @ (Bilat) Frontal
  - Wants to discuss current headache
  - Hx of similar headaches
  - Pressure/tightness
  - Worse in the AM
  - Severity: Moderate
  - Frequency > 3 per week
  - Episodes last less than an hour
  - Aggravating: weather changes
  - Alleviating: analgesics
- **Nasal/Sinus congestion**

Ø Nausea / Vomiting
Ø Photophobia / Vision Problem(s)
Ø Arm Weakness / Dysarthria/Speech impairment / Facial Weakness / Leg Weakness / Aura / Ataxia / Vertigo / Face pain
Ø Fever

Other Data

Cause	Ø Result of Injury
Tests/Consults	Ø Imaging
Gyn Hx	Ø Pregnancy
PMHx	• **Fe Deficiency** • **Chronic Sinusitis** Ø Migraine Ø Brain Surgery
MEDs	• **Naproxen** Ø Analgesics Ø Abx Ø Nasal Decongestant Spray
Drug Allergies	• **Penicillin**
Substances	• **Smoker (0 - 1/2 PPD)**
Other	Ø Analgesics > 15 days/month

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Advanced Technology

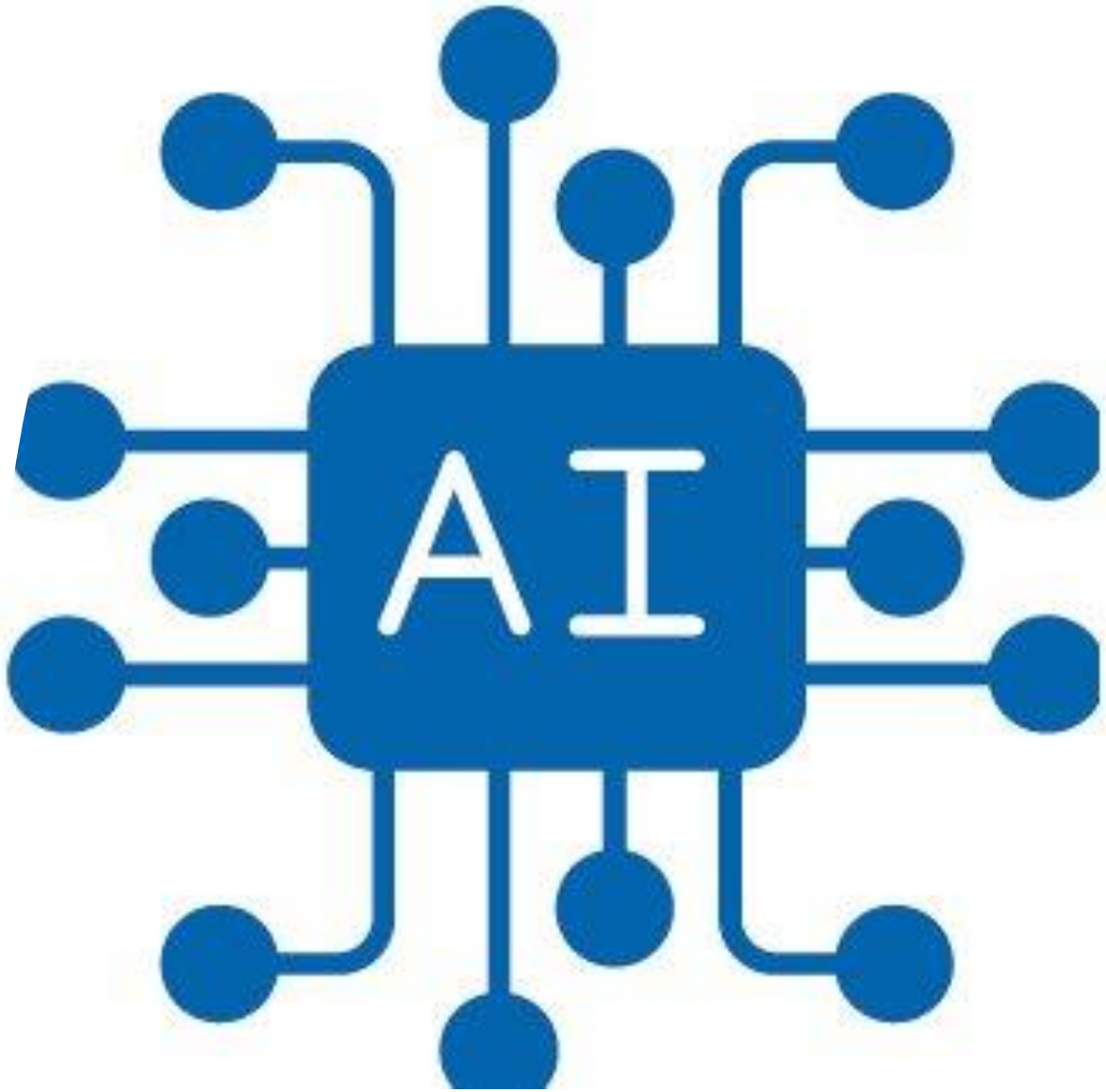
Leverage advanced technologies to enhance patient experience, and MDs ability to deliver better quality care.

Ai-driven efficiency

Reduced redundancy, data consistency

Smart data filtering

Patient-centric interface



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Broad Clinical Coverage

Most extensive and adaptable library of history questions.

Customizable for any specialty or clinical setting.

Incorporates **clinical filters** and tools to speed diagnosis and care.

11,000+ History Questions

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Automation

Automate patient intake and clinical workflows to increase efficiency, information sharing, collaboration, and productivity.

Optimized coding, billing.

Improved patient outcomes.

Enhanced compliance.

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Structured Data

Capture a broad and deep spectrum of structured patient data while accommodating individual preferences.

Enhanced decision-making.

Improved care coordination.

Data-driven insights.

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Data Sharing

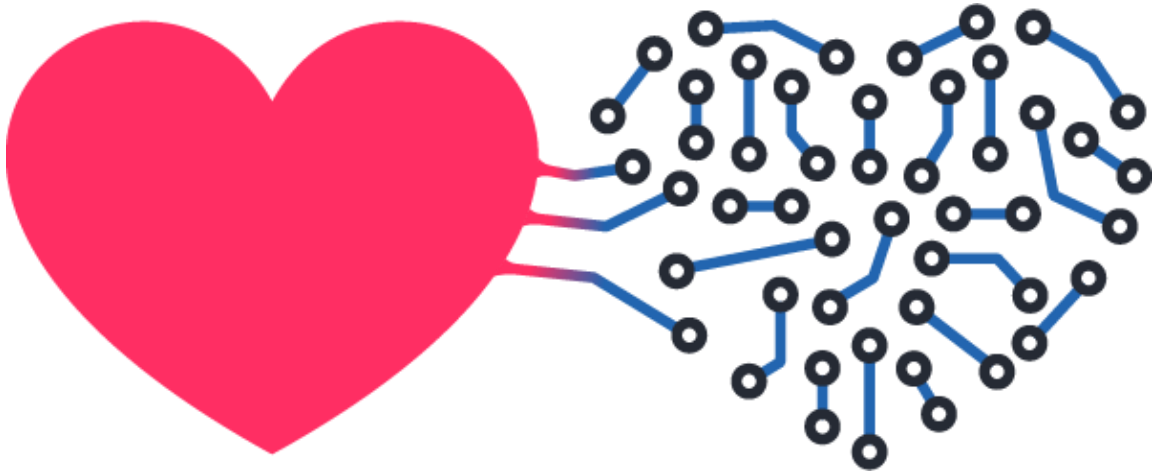
Standardized process for collecting structured medical history data for sharing across the primary and subspecialty care teams.

Improved care continuity.

Enhanced clinical collaboration.

Optimized resource use and triage.

Accelerated innovation.



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Patient Insights

Accurate patient insights, proven accuracy, informs personalized care for better outcomes

90%+ satisfaction
Patients feel their story has been heard
Patients spend **less than 5 minutes** to complete digital history.
Improves patient-MD communication.

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Integration & Interoperability

Open APIs allow for seamless data flow

Scalable across networks, platforms

Wide integration capacity reduces costs

Future-proofing, easily adapts to evolving technology with Agile processes

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Available in Multiple Languages

Histories are available in **English, Spanish, French and Arabic**

Growing number of languages – ask us!

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Mental Health-Focused Histories

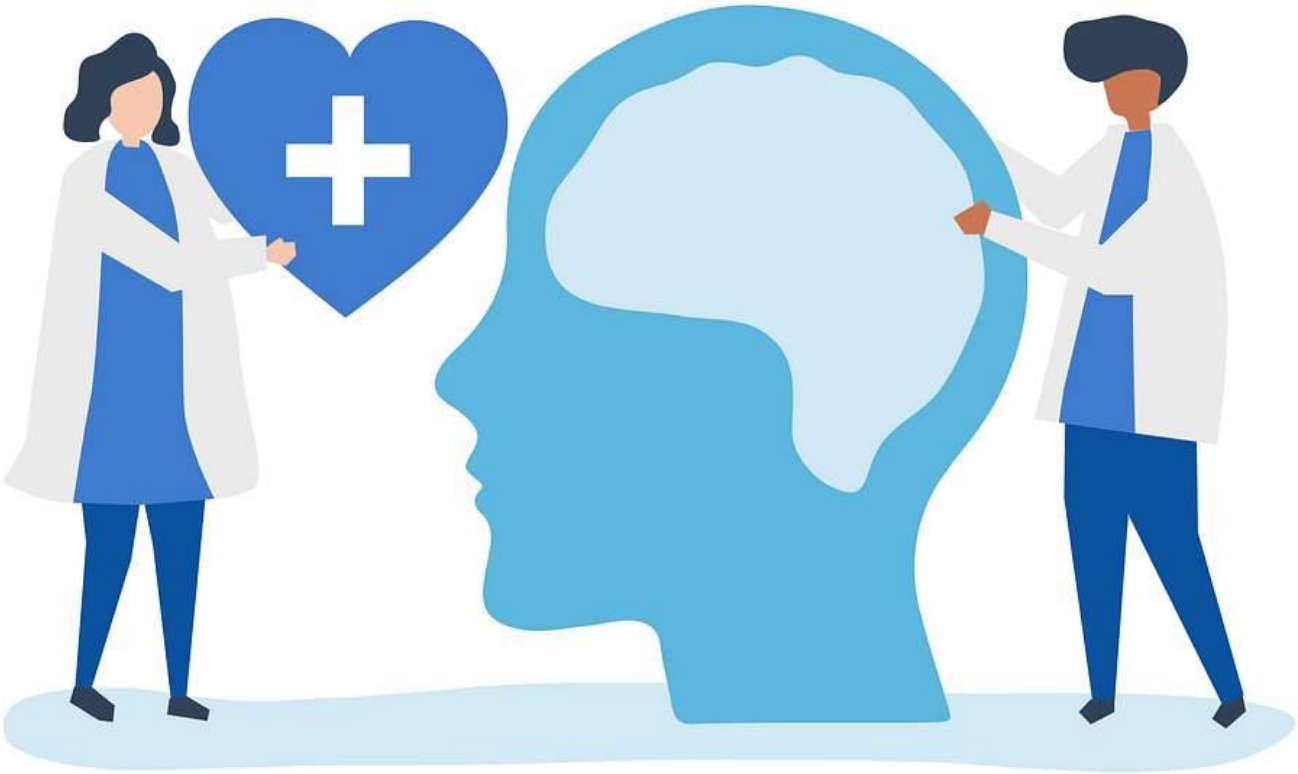
Mental Health medical histories provide you with best-in-class documentation.

Provides comprehensive picture of symptoms, experience of treatment.

Simplifies medication management.

Tracks compliance, improves patient engagement.

Evidence-based scores identify conditions and risks early, improving recognition.



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Medical History at your Fingertips

Note is available in your patient file/EMR.

Flexible, modifiable history content.

MDs have immediate access to Note.

Personalized care planning easier.

Enhanced patient safety: medications, allergy profile

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The ROI

Saves 25-50% of visit time, enhancing overall efficiency and reducing cost

Provides new data sets for analysis, optimizing treatment plans and decision making

Improves patient preparedness and satisfaction with medical visits

Web based no app to install or configure

24/7 Support Desk

No training, SmartCare360 can be learned in minutes.

Benefits to Your Organization

25-50% improved efficiency

Easier process, **less burnout**

Decreases documentation burden

Higher quality patient interactions

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SmartCare360 Filters

Smart filters for:
Clinical trial eligibility
GDMT compliance
Capture of CMS codes

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Screens for Social Determinants of Health (SDoH)

ICD-10 Codes that capture SDoH are fully covered by the SmartCare360 primary care intake.

Z55 – Problems related to education and literacy
- Z55.5 – Less than a high school diploma (Added, Oct. 1, 2021)
- NEW Z55.6 – Problems related to health literacy

Z56 – Problems related to employment and unemployment

Z57 – Occupational exposure to risk factors

Z58 – Problems related to physical environment (Added, Oct. 1, 2021)
- Z58.6 – Inadequate drinking-water supply (Added, Oct. 1, 2021)
- NEW Z58.8 – Other problems related to physical environment
  - NEW Z58.81 – Basic services unavailable in physical environment
  - NEW Z58.89 – Other problems related to physical environment

Z59 – Problems related to housing and economic circumstances
- Z59.0 – Homelessness (Updated)
  - Z59.00 – Homelessness unspecified (Added, Oct. 1, 2021)
  - Z59.01 – Sheltered homelessness (Added, Oct. 1, 2021)
  - Z59.02 – Unsheltered homelessness (Added, Oct. 1, 2021)
- Z59.1 – Inadequate Housing (Updated)
  - NEW Z59.10 – Inadequate housing, unspecified
  - NEW Z59.11 – Inadequate housing environmental temperature
  - NEW Z59.12 – Inadequate housing utilities
  - NEW Z59.19 – Other inadequate housing
- Z59.4 – Lack of adequate food (Updated)
  - Z59.41 – Food insecurity (Added, Oct. 1, 2021)
  - Z59.48 – Other specified lack of adequate food (Added, Oct. 1, 2021)
- Z59.8 – Other problems related to housing and economic circumstances (Updated)
  - Z59.81 – Housing instability, housed (Added, Oct. 1, 2021)
    - Z59.811 – Housing instability, housed, with risk of homelessness (Added, Oct. 1, 2021)
    - Z59.812 – Housing instability, housed, homelessness in past 12 months (Added, Oct. 1, 2021)
    - Z59.819 – Housing instability, housed unspecified (Added, Oct. 1, 2021)
  - Z59.82 – Transportation insecurity (Added, Oct. 1, 2022)
  - Z59.86 – Financial insecurity (Added, Oct. 1, 2022)
  - Z59.87 – Material hardship due to limited financial resources, not elsewhere classified (Added, Oct. 1, 2022; Revised, April 1, 2023)
  - Z59.89 – Other problems related to housing and economic circumstances (Added, Oct. 1, 2021)

Z60 – Problems related to social environment

Z62 – Problems related to upbringing
- Z62.2 – Upbringing away from parents
  - NEW Z62.23 – Child in custody of non-parental relative (Added, Oct. 1, 2023)
  - NEW Z62.24 – Child in custody of non-relative guardian (Added, Oct. 1, 2023)
- Z62.8 – Other specified problems related to upbringing (Updated)
  - Z62.81 – Personal history of abuse in childhood
    - NEW Z62.814 – Personal history of child financial abuse
    - NEW Z62.815 – Personal history of intimate partner abuse in childhood
  - Z62.82 – Parent-child conflict
    - NEW Z62.823 – Parent-step child conflict (Added, Oct. 1, 2023)
  - Z62.83 – Non-parental relative or guardian-child conflict (Added Oct. 1, 2023)
    - NEW Z62.831 – Non-parental relative-child conflict (Added Oct. 1, 2023)
    - NEW Z62.832 – Non-relative guardian-child conflict (Added Oct. 1, 2023)
    - NEW Z62.833 – Group home staff-child conflict (Added Oct. 1, 2023)
  - Z62.89 – Other specified problems related to upbringing
    - NEW Z62.892 – Runaway [from current living environment] (Added Oct. 1, 2023)

Z63 – Other problems related to primary support group, including family circumstances

Z64 – Problems related to certain psychosocial circumstance

Z65 – Problems related to other psychosocial circumstances

FACT: Time to capture SDoH with SmartCare360 = 0 minutes

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Enhancing GDMT Compliance and Tracking

Real-time capture.

Automated, complete documentation vs. Standards.

Easier recognition of GDMT in patient history.

FRACTION (HFREF)

GDMT has proven to reduce mortality and morbidity in patients with HFrEF (EF < 40%)

Drug classes include RAAS-I/ARNI, BB, MRA, and SGLT2-I

RAAS-I / ANRI

Entresto (sacubitril-valsartan) has shown to reduce all-cause death by 16% vs ACE-i

May consider RAAS-I (lisinopril, losartan, etc), switch to ARNI when clinically stable

BETA BLOCKERS

Beta blockers have been associated with a reduction in all-cause mortality by ~35%

Specific BB: bisoprolol, carvedilol, and metoprolol succinate

MRA

Spirinolactone may reduce the incidence of cardiovascular death and heart-failure associated hospitalization

Initiate after RAAS-I/ARNI and BB

Adapted from Dixit et al. US Cardiology Review 2021;15:e07.

SGLT2-I

Dapagliflozin and empagliflozin have shown to reduce all-cause death by 13% and hospitalizations

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